



Hospital Wide Policy

Nutrition and Hydration

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Hospital-Wide	All Staff

Introduction

WA Health has a duty of care to ensure all inpatients have access to safe, appropriate and adequate food and fluids to meet their requirements during their hospital admission.

Sir Charles Gairdner and Osborne Park Health Care Group (SCGOPHCG) staff are committed to the prevention, identification and management of hospital acquired malnutrition and identification and management of pre-existing (chronic) malnutrition.

Risk Statement

Non-compliance with this policy

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☒
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

Nutrition	Adequate energy and nutrients to meet the body's needs and promote good health
Hydration	Adequate water in the intracellular and extracellular compartments of the body ¹
Fluid Balance	The balance of the input and output of fluids in the body to allow metabolic processes to function correctly ²
Malnutrition	A state of deficiency, excess or imbalance of energy, protein and other nutrients, results in a measurable adverse effect on body composition, function and clinical outcomes
Malnutrition Screening Tool (MST)	A validated risk screening tool used either as a standalone tool or performed as part of the Waterlow Risk Assessment Tool, to identify any patients with or at risk of malnutrition
Mini Nutritional Assessment (MNA)	A validated nutritional screening and assessment tool for use in older adults to identify malnutrition or risk of malnutrition

Policy Principles

This Policy establishes the minimum standards for the screening, assessment and management of nutrition and hydration at SCGOPHCG and is aligned with the current Nutrition Standards for Adult Inpatients in WA Hospitals.

Compliance with this policy contributes to compliance with National Safety and Quality Health Service (NSQHS) Standard 5: Comprehensive Care.

1.0 Malnutrition Screening

Early detection of malnutrition and implementation of appropriate nutrition support reduces the risk of a patient's nutritional status deteriorating during an episode of care affecting their recovery / clinical outcome.

It is mandatory that each inpatient is screened within ten (10) hours of admission using the validated Malnutrition Screening Tool (MST) located on the Nursing Admission Assessment Form (NAAF). The patient's height and weight are essential to correct assessment and must be recorded.

Diet type, fluid comments, allergies, intolerances and MST score should be entered into the Automated Menu System (AMS) by nursing staff prior to the next meal service, to ensure the correct meal and fluid type are delivered as soon as possible.

Patients identified 'at risk' of malnutrition upon initial screening are managed according to the Malnutrition Action Plan within the NAAF.

The "Waterlow" Risk Assessment Tool within the NAAF is used to identify and rescreen patients at risk of malnutrition and documents the most recent MST score.

2.0 Assessment

2.1 Nutrition assessment

Upon referral, the Dietitian will undertake a comprehensive nutrition assessment to evaluate a patient's nutritional status and identify nutrition-related problems.

The appropriate oral, enteral or parenteral support will be planned and provided. Standardised enteral feeding protocols will be followed until a Dietitian assessment can be completed.

2.2 Hydration assessment

Medical and nursing staff should conduct an assessment of hydration on admission. Assessment may include medical history, clinical examination, current medications, clinical monitoring and laboratory results / investigations.

3.0 Management

3.1 Nutrition Care Planning

After assessment, the Dietitian will document a Nutrition Care Plan (NCP) in the medical record. This NCP will include suggested food and fluids to meet therapeutic requirements and the nutritional and/or medical/nursing monitoring required.

3.2 Hydration

Hydration therapy may be provided via oral, intravenous/parenteral, subcutaneous or enteral routes as clinically indicated. Therapy may include prescription of artificial hydration, including intravenous and subcutaneous fluids. The purpose of the therapy will be documented by the Medical Officer i.e. for maintenance, resuscitation or replacement fluids¹. Nursing staff are responsible for ensuring the therapy is delivered and results of same documented.

The purpose, route of administration, fluid type and volume should be assessed and regularly reviewed based on current clinical presentation to prevent dehydration, fluid overload, acidosis, and their associated complications under medical prescription by the treating team

Fluid Restriction (FR) volume is determined by the medical team. A FR sign, with the numerical restriction value, should be visible within the patient's room or board and the fluid restriction amount must be entered into the AMS by nursing staff. Additional fluids offered by catering between and during meal service such as water jugs, milk drinks, juice, tea and coffee will be restricted. Nursing staff will communicate with Food Service Assistants if extra fluids are allowed.

Monitoring of fluid balance will be recorded by nursing staff on the Fluid Balance Chart (MR904) as per [Nursing Practice Guideline 25](#).

3.3 Provision of food, fluids and nutrition supplements

The hospital menu provides a wide range of therapeutic diets to meet each patient's nutritional requirements including texture modified foods and thickened fluids.

Individualised meal plans may also be provided by the Dietitian for patients whose needs cannot be met by the standard menu and therapeutic diets available on the AMS.

Enteral or parenteral nutrition should be considered if a patient's requirements cannot be met through oral intake alone or if oral intake is contraindicated.

Ensuring each patient receives safe, appropriate and adequate food and fluids is the responsibility of the multidisciplinary team, including but not exclusive to Catering, Dietetics and Nutrition, Medical, Nursing, Speech Pathology and Occupational Therapy.

This team is responsible for:

- Considering the patient's physical, physiological and psychological ability to eat
- Considering the patient's psychosocial, cultural and religious needs
- Considering drug-nutrient interactions and biochemical / haematological / microbiological markers in treatment planning
- Monitoring and documenting oral food, fluid and enteral / parenteral nutrition intake
- Assisting patients with meal selection, meal set-up and/or feeding assistance considering flexible meal timing and meal service arrangements
- Providing medical nutrition supplements
- Providing texture modified foods and fluids in accordance with IDDSI standards.
- Providing thickening products to the wards for preparation of thickened beverages for inpatients
- Providing a menu compliant with the current Nutrition Standards for Adult Inpatients in WA Hospitals
- Educating and advising the patient and their family/carers regarding specialized food and fluid requirements
- Ensuring patients are not nil by mouth for more than three (3) consecutive days and fasting frequency is minimised whenever possible

3.4 Feeding Assistance Required (FAR) and mealtime environment

All staff should focus on providing a mealtime environment that is pleasant and relaxed to enhance consumption and enjoyment of meals. Mealtime interruptions from hospital routines, clinical procedures and ward rounds should be minimised when possible.

Level of feeding assistance is determined according to patient diagnosis, manual dexterity and level of cognition. Nursing staff must document the level of assistance required by patients in the nursing care plan – eating independently, set-up only, needs help with eating. Full or partial mealtime assistance should be provided to patients who have

difficulties with self-feeding. Independence with eating and drinking should be promoted and patients can be provided with appropriate feeding aids such as adaptive aids, cutlery and drinking devices if required and ordered through the AMS by nursing staff under 'Eating Aids'.

4.0 Discharge planning

The Dietitian, together with the multidisciplinary team, is responsible for providing a nutrition and hydration care plan prior to discharge where possible or within 48 hours of discharge for patients requiring ongoing nutrition and / or hydration support.

The Dietitian must ensure the plan;

- Is clearly communicated to the patient, family and/or carer
- Includes relevant education on food and fluid requirements with provision of supporting materials
- Identifies any required services or referral to community services, including contact details and time frame for follow-up
- Includes registration to the Home Enteral Nutrition (HEN) program (if patient meets eligibility criteria) with information on access to feeding equipment and consumables provided

Compliance, monitoring and evaluation

The SCGOPHCG Nutrition and Hydration Working Group will undertake compliance monitoring and evaluation of this policy on an annual and as needed basis.

The requirements of this policy will be assessed as part of the Hospital Audit program CENSAs and via annual hospital wide malnutrition prevalence audits and MST compliance audits.

Related Documents

- [Nutrition Standards for Adult Inpatients in WA Hospitals](#)
- [SCGH Management of Food Allergies and Intolerances Policy](#)
- [SCGH Discharge of Patient Policy](#)
- [SCGH Medication Management Guideline High Output Stoma](#)
- [SCGH Intensive Care Unit \(ICU\) and General High Dependency Unit \(GHDU\) weekend and after-hours enteral feeding protocol including the management of refeeding syndrome](#)
- [SCGH Oral and Enteral Nutrition Refeeding Syndrome Guideline](#)
- [SCGH Initial Enteral Feeding Regimen](#)
- [SCGH Post-Operative Enteral Feeding Guideline for Upper Gastrointestinal \(UGI\) Patients](#)
- [SCGH Nursing Practice Guidelines - Nursing documentation \(#25\) fluid balance chart](#)
- [OPH Nutrition and Dietetics](#)
- [WA Health Nutrition Standards for Adult Inpatients in WA Hospitals](#)
- [OPH Nursing and Midwifery Practice Guidelines Gastrostomy Tube \(PEG\)](#)
- [OPH Nursing and Midwifery Practice Guidelines Nasogastric Tube](#)
- [OPH Hospital Wide Guideline Pressure Injury Prevention and Management](#)
- [IDDSI Framework](#)

References

1. [Dietitians Association of Australia. Nutrition and Hydration Policy Support Handbook for Acute Adult Inpatient Setting. February 2019](#)

2. Scales K, Pilsworth J (2008) The importance of fluid balance in clinical practice. *Nursing Standard*. 22, 47, 50-57.
3. [Agency for Clinical Innovation \(ACI\). Nutrition standards for adult inpatients in NSW hospitals. Chatswood, NSW: ACI Nutrition Network; 2011.](#)

Authorisation & Version Control

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For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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